



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : قمر فتحى بشير طحاوي
المحافظة : الجيزة
الحى / المدينة : الاخصاص
رقم البطاقة : 26707142100589 رقم التليفون : 01020241228
العمر (تاريخ الميلاد) : 14-07-1967 المهنة : ربه منزل
العنوان : -----
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: قمر فتحي بشير طحاوي National ID: 26707142100589	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



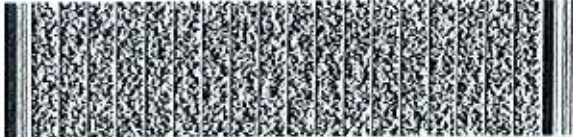
قمر
فتحي بشير طحاوي
٢ ش سعد زغلول
الحوامدية - الجيزة

١٩٦٧/٠٧/٢٤ ٢٦٧٠٧١٤٢١٠٠٥٨٩
KR6435423

٢٠٢٤/٠٨ ٢٦٧٠٧١٤٢١٠٠٥٨٩
ربة منزل

أنثى
مسلمة
متزوجة
عبد الناصر شحات محمد حسن

٢٠٢١/٠٨/٠٦ البطاقة مارية حتى ١

04/07/2026

MDT FORM

Name: بشير طحانى Fathy امر	MRN: 7892
Age:	ID: 26707142100589
Menopausal Sta POST	Phone: 010241228
NRS SCORE:	Marital Status:
BMI:	Family History: -VE
CO Morbidity: HTN	Reason Of MDT: NEW CASE

Clinical Data:

BX: LT. IDC G2
ER: 8/8 PR: 8/8 HER2: 0 KI67: 70%

R

L

Sonomammography:

15/2/2025 LT. LOQ 4'O'CLOCK 2.6*2.3*2.8 CM ISEPARABLE FROM THE SKIN

LT. AXILLARY LARGE LN WITH THICKENED CORTEX 2.4*1.4 CM BIRADS 5

Mets Workup:

CT CAP>>>L4 VERTEBRAL BODY LYTIC FOCUS>>>BONE SCAN>>>>FREE

History:

CLINICALLY SKIN DIPPLING OVER MASS WITH LARGE AXILLARY LN

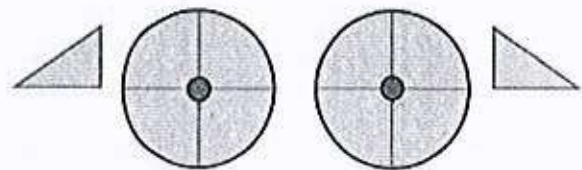
CT4 N2 M0

Recommendations:

for nact and clipping for possiple cbs and alnd

MDT Outcome & Plan

- Portacath Insertion
- clipping for possiple It cbs and alnd (Include skin over the mass)
- NACT (4 AC + 4 Docetaxel)



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد القنوي	د/ ابو بكر عوازي			

Name : Qamar Fathy Bashir Tahawi

Branch : Dokki

Ref.Physician Prof : Goustav Roussey Egypt

٧ ٨٩٢
Date : 16-03-2026 09:25

Medical No. : 333154

ISOTOPIC BONE SCAN REPORT

- **Indication:** 58-year-old female with history of breast carcinoma. Evaluation for possible osseous metastatic disease.
- **Imaging Protocol:** Whole-body planar images (anterior and posterior projections) acquired approximately 2–3 hours post intravenous injection of the tracer (Tc-99m MDP).

FINDINGS:

- No abnormal radiotracer uptake suggestive of skeletal metastatic disease is identified.
- Degenerative/Arthritic changes are noted in the spine (especially lumbar), both shoulders, and weight-bearing joints.

IMPRESSION:

- No definite scintigraphic evidence of osseous metastases.
- Follow up as per clinical pathway.

Much oblige
Dr. Alaa Nagy.
Nuclear medicine

٧٨٩٩

Patient Name : الأستاذ / د. محمد بشير طحاروي	Request No. : 13004255
Patient No. : 26707142100589	Request Date : 29/12/2025
Path No. : 4035/25	Verification Date : 11/01/2026
Sex : Female	Financial Category : جوستاف روسي - مبادر
Age : 58 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data

Left breast mass.

Nature of specimen

Tru cut biopsy collected on 29/12/2025, at 12:00, cold ischemia time :10 seconds.

Gross examination

Fragments of grey white tissue core ranging in size from 0.5 cm up to 1 cm, totally submitted.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counterstain. The sections treated against:
HER2/NEU (4B5) Ki67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Absent
- * Lobular intra epithelial neoplasia : Absent
- * Angiolysis: Absent.
- * Microcalcification : Absent
- * Perineural invasion: Absent
- * IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 70 % tumor cells (Allred score 3/3)
- * PR: Strong nuclear reaction in 90% tumor cells (Allred score 3/3)
- * HER2: No membrane reaction in tumor cells (score 0)
- * Ki 67: 70 %

Verified by:

Dr. Heba Hassan

Printing Date : 11/01/2026 10:31:40

Page 1 of 1

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شعبة رقم (4) - الدور الأول - مبنى رقم (987) - شبراخيت الأولى - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Alisir El Qadema - Cairo, Egypt

Patient Name : الأستاذ / د. يحيى بلال طحاري	Request No. : 13004255
Patient No. : 26707142100589	Request Date : 29/12/2025
Path No. : 4035/25	Validation Date : 11/01/2026
Sex : Female	Financial Category : جوستاف رومبي - مبالغه
Age : 58 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Diagnosis

LEFT BREAST MASS, TRU CUT BIOPSY

-Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)
(according to Nottingham grading system of breast cancer)

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 8/8).
- * HER2: Negative (score 0)
- * Ki67: 75 %
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010. Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof.Dr.Maha Guimei
Dr.Heba El-Shafey

Verified by :

Dr.Heba Hegazy



Patient Name : الإسماء / أم أحمد بشير ممدوح	Request No. : 13004256
Patient No. : 26707142100589	Request Date : 29/12/2025
Path No. : 4035/25	Verification Date : 11/01/2026
Sex : Female	Financial Category : جوستاف روسي - مباحرة
Age : 58 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data

Left breast mass.

Nature of specimen

Tru cut biopsy collected on 29/12/2026, at 12:00 , cold ischemia time :10 seconds .

Gross examination

Fragmented grey white tissue core ranging in size from 0.5 cm up to 1 cm, totally submitted .

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. IHC staining treated against:
HER2/NEU (4B5) Ki67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Absent
- * Lobular intra epithelial neoplasia : Absent
- * Angiolinvasion: Absent
- * Microcalcification :Absent
- * Perineural invasion: Absent
- IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 70 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 90% tumor cells (Allred score 8/8)
- * HER2: No membrane reaction in tumor cells (score 0)
- * Ki 67: 70 %

Verified by :

Dr. Heba Hassan

Printing Date : 23/01/2026 10:31:40

Page 1 of 1

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - MIsr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / د. محمد علي محمد	Request No. : 15004255
Patient No. : 28787142100589	Request Date : 29/12/2025
Path No. : 4085/25	Validation Date : 11/01/2026
Sex : Female	Financial Category : جوستاف رومي - مبالغه
Age : 60 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Diagnosis

LEFT BREAST MASS, TRU CUT BIOPSY

-Invasive duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2)
(according to Nottingham grading system of breast cancer)

IPT:

- * ER: Positive (score 8/8)
- * PR: Positive (score 8/8)
- * HER2: Negative / score 0
- * Ki67: 70 %
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2013. Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Maha Guirgis
Dr. Heba El-Shafawy

Verified by:

Dr. Heba El-Shafawy

Printing Date: 21/01/2026 11:51:13

Page 2 of 3

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CAP
ACCREDITED

شركة راسم (د) - الدور الاول - مبنى رقم (4) - شارع القاهرة - القاهرة - مصر
Nasr No. (4) - 1st Floor - Building No. 987 - Nile Corridor - El-Qadima - Cairo, Egypt

Name : Qamar Fathy Bashir

Date : 04-01-2026 10:07

Branch : Maadi

Medical No. : 288100

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : Walkin

**DIGITALIZED LOW DOSE SOFT TISSUE AMMOGRAPHY WITH
COMPLEMENTARY BREAST US EXAMINATION OF BOTH
BREASTS REVEALED:**

- ACR A: predominant fatty breast parenchyma.
- A clustered hyperdense lesion is seen at the LOQ of the left breast corresponding by US to an irregular globular shaped heterogeneous hypoechoic lesion is seen at 5 o'clock surrounded by echogenic halo. It measures 3 x2.2 cm.
- Left axillary suspicious lymph node showing thickened cortex and effaced hilum. It measures 6x7 mm.
- No pathological microcalcification clusters.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- Right non specific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Left breast UOQ suspicious lesion and suspicious LN. (BIRADS V).

Much Obligated;
Dr. Sarah Gamal, MSc.

Dr. Sara Gamal

Name : kamar fathy besher

Branch : Portable

Ref.Physician Prof : cap+c

Date : 04-03-2026 02:24

Medical No. : GRI-26707142100589

Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

Known case of breast CA with no available previous films for comparison.

I- CHEST:

- Left breast lesion is seen measuring 29 x 21 mm for sono-mammography.
- Few left axillary lymph nodes are seen largest measuring 29 x 28 mm.
- Few tiny bilateral scattered pulmonary nodules largest measuring 5 mm at the right middle lobe.
- Diffuse mosaic attenuation of both lower lobes ? small airway disease for clinical correlation.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral elastofibroma dorsi more evident on the right side.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Few bilateral subcentimetric external iliac lymph nodes.
- No ascites.
- Tiny L4 vertebral body lytic focus.

Opinion:

- ❖ Left breast lesion with left axillary lymphadenopathy for sono-mammography.
- ❖ Few tiny bilateral scattered pulmonary nodules ? inflammatory for follow up.
- ❖ Tiny L4 vertebral body lytic focus for bone scan.

Dr. Sherif Wagdy

Dr. Sherif Wagdy